# Anastrozole · daily hormone therapy

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

| Check | Before starting | First weeks | During treatment | Longer-term review | Change / completion |
|---|---|---|---|---|---|
| **Bloods / symptom review** | full blood count (FBC), renal and liver profile | Review tolerability and medicines | Further tests when clinically indicated | Review symptoms, adherence and results | Follow unresolved abnormalities |
| **electrocardiogram (ECG) / echo** | No routine ECG / echo for this medicine alone | Investigate new cardiac symptoms | Further cardiac tests by risk / symptoms | Companion-drug cardiac rules remain separate | Follow abnormalities when indicated |
| **Bone density / dual-energy X-ray absorptiometry (bone density) (DXA)** | Adjuvant aromatase inhibitor (AI): baseline DXA unless already on adjuvant bisphosphonates | Review previous DXA and fracture risk | No cycle-based DXA appointments | Lower-risk AI: reassess in 1–2 years; individualise | Continue bone care according to risk |
| **Early-breast surveillance** | Confirm early / curative setting | Set first follow-up mammogram date | Assess new symptoms promptly | Annual mammography of remaining breast tissue | No routine computed tomography (CT) / markers if asymptomatic |
| **Advanced-disease response** | Document disease sites / baseline assessment | New symptoms can bring assessment forward | Generally every 2–4 months; individualise | Adapt interval and imaging to disease / benefit | Markers supplement, never replace, assessment |

THE WHOLE TREATMENT JOURNEY

# Anastrozole · daily hormone therapy: treatment and checks

### Postmenopausal hormone-sensitive breast cancer

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First weeks | During treatment | Longer-term review | Change / completion |
|---|---|---|---|---|---|
| **Treatment plan** NCCP 00254 | Confirm indication and prescription | Start the agreed daily / injection schedule | Continue unless prescriber changes the plan | Review duration, benefit and tolerability | Agree continuation, switch or completion |
| **Bloods / symptom review** NCCP 00254 | FBC, renal and liver profile | Review tolerability and medicines | Further tests when clinically indicated | Review symptoms, adherence and results | Follow unresolved abnormalities |
| **ECG / echo** NCCP 00254 | No routine ECG / echo for this medicine alone | Investigate new cardiac symptoms | Further cardiac tests by risk / symptoms | Companion-drug cardiac rules remain separate | Follow abnormalities when indicated |
| **Bone density / DXA** NCCP 00254 · AIBL2025 | Adjuvant AI: baseline DXA unless already on adjuvant bisphosphonates | Review previous DXA and fracture risk | No cycle-based DXA appointments | Lower-risk AI: reassess in 1–2 years; individualise | Continue bone care according to risk |
| **Early-breast surveillance** NCCP 00254 | Confirm early / curative setting | Set first follow-up mammogram date | Assess new symptoms promptly | Annual mammography of remaining breast tissue | No routine CT / markers if asymptomatic |
| **Advanced-disease response** NCCP 00254 | Document disease sites / baseline assessment | New symptoms can bring assessment forward | Generally every 2–4 months; individualise | Adapt interval and imaging to disease / benefit | Markers supplement, never replace, assessment |
| **Early: radiotherapy** ESMO_EBC2024 | Review surgery / pathology | Plan radiotherapy if indicated | Coordinate endocrine timing | Follow the early-breast plan | Review late effects when relevant |

# What each check is for

1 **Confirm the plan**

Before starting or changing treatment

Sources: NCCP 00254

2 **Checks and symptoms**

At the agreed review points

Sources: NCCP 00254

3 **Review ongoing benefit**

At setting-specific decision points

Sources: NCCP 00254

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.20 · Sources checked 2026-10-25

Source reconciled; independent clinical, pharmacy and patient/PPI review pending